

Patient Name : K V V S N REDDY

Age / Gender : 47 years / Male

Patient ID : 20138

Source : DIRECT

Referral : SELF

Collection Time : Nov 22, 2024, 06:15 a.m.

Receiving Time : Nov 22, 2024, 06:15 a.m.

Reporting Time : Nov 22, 2024, 09:09 a.m.

Sample ID :



9243270002

Test Description	Result(s)	Biological Reference Intervals	Unit(s)
POST PRANDIAL PLASMA GLUCOSE			
Post Prandial Plasma Glucose Method : Glucose oxidase/Peroxidase	159.34	< 180	mg/dL
Instrument:Done on BIOSYSTEMS BA 400, a Fully Automated Biochemical Analyser			

Note

HbA1c

HbA1c Method:HPLC (BIORAD D 10)(Sample Type:Blood)	5.3	Non Diabetic : 4.0 - 5.6 % Border line : 5.7 - 6.4 % Good control in Diabetes : < 6 Years : < 8.5 % 6 Years - 12 Years : < 8.0 % 13 Years - 18 Years : < 7.5 % > 18 Years : < 7 %	%
Estimated Average Glucose (eAG) Calculated	105.41		mg/dL

Interpretation :

- HbA1c is used for monitoring diabetic control. It reflects the estimated average glucose (eAG).
- HbA1c has been endorsed by clinical groups & ADA (American Diabetes Association) guidelines 2012, for diagnosis of diabetes using a cut-off point of 6.5%. ADA defined biological reference range for HbA1c is 4% to 5.7%. Patients with HbA1c value between 5.7% to 6.5% are considered Pre-diabetic.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test.
- Low glycated haemoglobin(below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases,chronic anaemia(especially severe iron deficiency &haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
- To estimate the eAG from the HbA1C value, the following equation is used: $eAG(mg/dl) = 28.7 \times A1c - 46.7$
- Interference of Haemoglobinopathies in HbA1c estimation.
 - For HbF > 25%, an alternate platform (Fructosamine) is recommended for testing of HbA1c.
 - Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status
 - Heterozygous state detected is corrected for HbS and HbC trait.Hemoglobin electrophoresis (HPLC method) is recommended for detecting hemoglobinopathy.

Note

URINE ALBUMIN/CREATININE RATIO			
*URINE ALBUMIN Latex Method(Sample Type:Urine)	25.55	0 - 15	mg/L
*URINE CREATININE Jaffe's Compensated(Sample Type:Urine)	92.51	37 - 250	mg/dL

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Test Description	Result(s)	Biological Reference Intervals	Unit(s)
*URINE ALBUMIN/CREATININE RATIO calculated	27.62	0 - 30	mg/gram creatinine

Instrument: Tests performed on Biosystems BA 400, A Fully Automatic Biochemistry analyser

Normal: 0 - 30

Microalbuminuria: 30 - 300 mg/gram creatinine

Macroalbuminuria : > 300 mg/gram creatinine

Note

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VITAMIN B12

*Vitamin B12	116	222-1439	pg/mL
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CLIA

Method : CLIA ChemiLuminescence Immunoassay

Method: ChemiLuminescence Immunoassay(Serum)

Interpretation:

- Vit B12 levels are decreased in megaloblastic anemia, partial/total gastrectomy, pernicious anemia, peripheral neuropathies, chronic alcoholism, senile dementia, and treated epilepsy.
- An associated increase in homocysteine levels is an independent risk marker for cardiovascular disease and deep vein thrombosis.
- HoloTranscobalamin II levels are a more accurate marker of active VitB12 component.

Note: Patients on Biotin supplement may have interference in some immunoassays. With individuals taking high dose Biotin (morethan 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended.

Ref: Arch Pathol Lab Med—Vol 141, November 2017

Note

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URIC ACID

URIC ACID	9.25	Male:3.5-7.2 Female:2.6 - 6.0	mg/dL
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Method:URICASE/PEROXIDASE(Sample Type:Serum)

Note

LIPID PROFILE

TOTAL CHOLESTEROL	232.99	Desirable : Up to 200 mg/dL Borderline High: 200-239 mg/dL High : > 240 mg/dL	mg/dL
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Method:Cholestrol Oxidase/Peroxidase(Sample Type:Serum)

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Test Description	Result(s)	Biological Reference Intervals	Unit(s)
TRIGLYCERIDES Method:Glycerol phosphate oxidase/Peroxidase(Sample Type:Serum)	512.88	Normal : Up to 150 mg/dL Borderline High : 150-199 mg/dL High : 200-499 mg/dL Very high : > 500 mg/dL	mg/dL
HDL CHOLESTEROL Method:Direct(Sample Type:Serum)	35.12	> 40 mg/dL High Risk : < 35 mg/dL Low Risk : > 60 mg/dL	mg/dL
*LDL CHOLESTEROL Calculated	95.29	Optimal : Up to 100 mg/dL Near optimal/above optimal : 100-129 mg/dL Borderline High : 130-159 mg/dL High : 160-189 mg/dL Very High : > 190 mg/dL	mg/dL
*VLDL CHOLESTEROL Calculated	102.58	5 - 51 mg/dL	mg/dL
*CHOL / HDL RATIO Calculated	6.63	Low risk : < 3.5 Average risk : 3.5 - 5.0 High risk : > 5.0	
*LDL/HDL RATIO Calculated	2.71	Desirable : < 0.5-3.0 Borderline : 3.1- 6.0 Elevated : > 6.0	

INSTRUMENT: DONE ON BA 400 BIOSYSTEMS FULLY AUTOMATED BIOCHEMISTRY ANALYZER

Note

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Liver Function Tests

BILIRUBIN (TOTAL) DICHLOROPHENYL DIAZONIUM (Sample Type:Serum)	1.23	<24Hrs: 1.0 - 8.0 mg/dL <48Hrs: 6.0 - 12.0 mg/dL 3 - 5 Days: 10.0 - 14.0 mg/dL Adults : <1.0 mg/dL	mg/dL
*BILIRUBIN (DIRECT) DICHLOROPHENYL DIAZONIUM (Sample Type:Serum)	0.45	< 0.2 mg/dL	mg/dL
BILIRUBIN (INDIRECT)(Sample Type:Serum) calculated Method : Calculated	0.78	0.2 -0.8 mg/dL	mg/dL
SGOT IFCC without pyridoxal 5-P(Sample Type:Serum)	16.87	0 - 40	U/L
SGPT IFCC without pyridoxal 5-P(Sample Type:Serum)	23.00	0 - 41	U/L

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Test Description	Result(s)	Biological Reference Intervals	Unit(s)
TOTAL PROTEIN BIURET (Sample Type:Serum)	7.48	Pre mature : 0.6 -6.0 g/dL New born : 4.6 - 7.0 g/dL One week : 4.4 - 7.6 g/dL 7 months - 1 year : 5.1-7.3 g/dL 1 Year - 2 years : 5.6 - 7.5 g/dL > 3 years : 6.5 - 8.0 g/dL	g/dL
ALBUMIN BROMOCRESOL GREEN(Sample Type:Serum)	4.77	3.5 - 5.0 g/dL	g/dL
*GLOBULIN Calculated	2.71	2.5 - 4.5	g/dL
*A:G RATIO	1.76	1.5 - 3.5	
ALP (ALKALINE PHOSPHATASE) 2Amino 2Methyl - 1Propanol Buffer IFCC Method(Sample Type:Seru	100.31	<115 U/L	U/L
*gGT Method : IFCC	27.69	< 55	U/L

S.PHOSPHORUS

PHOSPHORUS Method: PHOSPHOMOLYBDATE/UV (Sample Type:Serum)	2.74	2.5 - 4.5	mg/dL
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Method:TEST DONE ON BIOSYSTEMS BA 400. A FULLY AUTOMATED BIOCHEMISTRY ANALYSER

CREATININE

CREATININE Method:JAFTE COMPENSATED (Sample Type:Serum)	1.02	0.7 - 1.2	mg/dL
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Instrument:Test Done On Fully Automated Biochemistry Analyser, BIOSYSTEMS BA 400

Note

RA FACTOR

RA FACTOR Method:Latex(Sample Type:Serum)	6.93	< 30 IU/mL	IU/ml
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Interpretation

The RF test must be interpreted in conjunction with a person's symptoms and clinical history.

In those with symptoms and clinical signs of rheumatoid arthritis, the presence of significant concentrations of RF indicates that it is likely that they have RA. Higher levels of RF generally correlate with more severe disease and a poorer prognosis.

A negative RF test does not rule out RA. About 20% of people with RA will have very low levels of or no detectable RF. In these cases, a CCP antibody test may be positive and used to confirm RA.

Positive RF test results may also be seen in 1-5% of healthy people and in some people with conditions such as: Sjogren syndrome, scleroderma, systemic lupus erythematosus (lupus), sarcoidosis,endocarditis, tuberculosis, syphilis, HIV/AIDS, hepatitis, infectious mononucleosis, cancers such

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as leukemia and multiple myeloma, parasitic infection, or disease of the liver, lung, or kidney The RF test is not used to diagnose or monitor these other conditions.

Note

"Test marked with * is under NABL scope.

S.IRON

IRON	113.07	Men : 65 - 175 µg/dL Women : 50 - 170 µg/dL	µg/dL
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Method:FERROZINE(Sample Type:Serum)

Instrument:Test Done On Fully Automated Biochemistry Analyser, BIOSYSTEMS BA 400

Note

UREA

UREA	15.60	Cord: 21 - 40 mg/dL Pre - Mature : 3 - 20 mg/dL New Born : 4 - 12 mg/dL Child : 5 - 18 mg/dL Adult : 15 - 39 mg/dL <60 Years : 8 - 23 mg/dL	mg/dL
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Method:UREASE / GLUTAMATE DEHYDROGENASE(Sample Type:Serum)

Note

FASTING PLASMA GLUCOSE

Fasting Plasma Glucose (F)	103.88	Desired Fasting Plasma Glucose:	mg/dL
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Method : Glucose oxidase/Peroxidase

For nondiabetic: 70 - 100 mg/dL

For Diabetic: 80 - 130 mg/dL

Thyroid Function Test

*T3	115.01	87 - 178	ng/dL
*T4	13.6	4.82 - 15.65	µg/dL
*TSH	0.086	0.38 - 5.33	µIU/mL

Method: Chemi Luminescence Immunoassay(Sample Type:Serum)

Interpretation

- Measurement of TSH in conjunction with T3,T4 or FT3,FT4 is used for assessment of thyroid gland functional status and also for monitoring patients on treatment for hyperthyroidism & hypothyroidism. Suppressed TSH (<0.01 µIU/mL) suggests a diagnosis of hyperthyroidism and elevated concentration (>7 µIU/mL) suggest hypothyroidism. TSH levels may be affected by acute illness and several medications including dopamine and glucocorticoids.TSH concentration are decreased (low or undetectable) in Graves disease. Increased in TSH secreting pituitary

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Test Description	Result(s)	Biological Reference Intervals	Unit(s)
adenoma (secondary hyperthyroidism); PPTH. TSH concentration are primary elevated in hypothyroidism (along with decreased T4) except for pituitary & hypothalamic disease.			
- Mild to modest TSH elevations in patient with normal T3 & T4 levels indicates impaired thyroid hormone reserves & incipient hypothyroidism (subclinical hypothyroidism). - Mild to modest decrease in TSH with normal T3 & T4 indicates subclinical hyperthyroidism. - Degree of TSH suppression does not reflect the severity of hyperthyroidism, therefore, measurement of free thyroid hormone levels is required in patient with a suppressed TSH level. - If free T4 is normal, free T3 should be checked as it is the first hormone to increase in early hyperthyroidism. Though TSH levels can also be used to effectively monitor patients being treated with thyroid hormones but it may be misleading. Therefore total or free T4 generally serve as front line assay during this period. The free T3 & T4 (FT3 & FT4) measures concentrations of free hormones, which are not affected by changing in concentrations of binding proteins, therefore more reliable indicator of true thyroid status.			
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HEMOGRAM (CBP)

*Haemoglobin	16.9	12.0 - 18.0	g/dL
*R.B.C count	5.86	3.80 - 5.30	10 ⁶ μL
*Leucocyte Count (WBC Count)	7500	4000 - 11000	μL
*Platelet Count	216000	120000 - 380000	μL
*PCV	52.0	36.0 - 56.0	%
*MCV	88.7	80.0 - 100.0	fL
*MCH	28.8	27-32	pg
*MCHC	32.5	32.0 - 36.0	g/dL
*RDW-CV	12.4	10.0 - 16.5	%
*Mean Platelet Volume (MPV)	7.4	5.0 - 10.0	fL
*PDW	16.9	12.0 - 18.0	%

***DIFFERENTIAL COUNT**

*Granulocyte	60.8	42 - 85	%
*Lymphocytes	35.9	11.0 - 49.0	%
*Monocytes	3.3	0.0 - 9.0	%
Comments:	Erythrocytosis.		

Instrument: TEST PERFORMED ON NIHON KOHDEN HEMATOLOGY ANALYZER

Note

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Electrolytes

Sodium	141.7	136 - 145	mmol/L
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Method: ISE Direct(Sample Type: Serum)

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Test Description	Result(s)	Biological Reference Intervals	Unit(s)
Potassium	4.87	3.3 - 5.1	mmol/L
Method: ISE Direct(Sample Type:Serum)			
Ionised calcium	5.23	4.5 - 5.3	mg/dL
Method: ISE Direct(Sample Type:Serum)			
Chloride	99.8	90 - 110	mmol/L
Method: ISE Direct(Sample Type:Serum)			

Instrument:Test Done On Fully Automated Biochemistry Analyser, Sensa Core ST-200

Note

ESR

*ESR	12	0 - 15	mm/Hour
Method	Automated Westergren		

Interpretation:

- It indicates presence and intensity of an inflammatory process. It does not diagnose a specific disease. Changes in the ESR are more significant than the abnormal results of a single test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis and polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

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COMPLETE URINE EXAMINATION

Colour	Pale yellow	Pale yellow
(Sample Type:Urine)		
pH	5.5	4.5 – 8.5
Specific Gravity	1.015	1.015 – 1.025
Glucose	Negative	Negative
Protein	Negative	Negative
Ketones	Negative	Negative
Bilirubins	Negative	Negative
Nitrite	Negative	Negative
Blood	Negative	Negative
Urobilinogen	Negative	Negative
Leukocytes	Negative	Negative

MICROSCOPIC EXAMINATION

Pus cells (WBCs)	2 - 3	0 - 2/hpf	/hpf
Epithelial cells	1 - 2	0 - 4/hpf	/hpf

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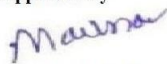
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Test Description	Result(s)	Biological Reference Intervals	Unit(s)
Red blood cells	Nil	0 - 2/hpf	/hpf
Cast	Nil	Nil	
Crystals	Nil	Nil	
Other	Nil	Nil	
Note			

END OF REPORT

Approved By

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